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Medicare Coverage Gaps Most Families Don't Know About

The hospital and rehab questions that cost families \$10K to \$30K

A plain-English coverage guide for adult children navigating a parent's hospital stay, rehab placement, or long-term care decision.

By Ryan Riggins

Riggins Strategic Solutions

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The call from the hospital almost never sounds like a coverage problem.

It sounds like good news. "Mom's stable, we're moving her to rehab." Or "We're keeping her another night for observation, just to be safe." Or "She'll need a skilled nursing facility for a few weeks while she gets stronger."

What the call doesn't tell you is that one wrong word in that conversation — *observation* instead of *inpatient*, or *Medicare Advantage* instead of *Original Medicare* — can cost the family \$10,000, \$20,000, or in the worst cases I've seen, \$40,000 out of pocket.

I'm not a Medicare specialist. I'm a real estate guy who has spent 8 years cleaning up after the coverage decisions families didn't know they were making. Half the homes I bought from senior families were being sold to pay for a rehab bill that *should* have been covered, but wasn't, because of a paperwork distinction nobody explained.

This guide is the conversation I wish someone had with every family before the 2 AM hospital call. It covers the five biggest coverage gaps, what to do *before* the next hospital stay, and the exact questions to ask so a \$30,000 surprise doesn't show up on the explanation of benefits.

Read it once now while everything's calm. Read it again the night Mom or Dad gets admitted. And if Mom's already in a coverage trap right now, jump straight to Section 6 — the appeals window closes faster than most families realize.

Three things, almost universally:

- 1. They think "Mom has Medicare" means coverage is automatic.** It is, until it isn't. Medicare has more carve-outs, exclusions, and status-based denials than most insurance plans. A hospital stay can end with full coverage or a 5-figure bill depending on what the chart says — not what care was actually delivered.
- 2. They confuse Original Medicare with Medicare Advantage.** These are two completely different products with completely different rules. Roughly half of Medicare beneficiaries are now on Medicare Advantage, and Medicare Advantage plans deny skilled nursing requests at a much higher rate than Original Medicare. If you don't know which one Mom has, find out today.
- 3. They learn about coverage gaps the day the bill arrives.** By then it's too late. Most gaps have appeal windows of 30 to 120 days, and most families miss them because nobody at the hospital tells them the clock has started. The five gaps in this guide all have one thing in common: you can prevent or fix them, but only if you act before the window closes.

This guide gives you the gaps, the windows, and the language. Use it.

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This is the single biggest Medicare coverage gap in America, and almost no family sees it coming.

Here's how it happens:

Mom falls. Ambulance takes her to the ER. The ER doctor decides she needs to stay — maybe she's confused, maybe her blood pressure is unstable, maybe they want to monitor her overnight. She gets a room. She gets a bracelet. She gets nurses checking on her. She gets meals. From every angle that matters to the family, she's a *patient in the hospital*.

But there are two ways the hospital can classify that stay:

- **Inpatient.** Admitted. Medicare Part A applies. Hospital stay is covered, and — critically — if she needs a Skilled Nursing Facility (SNF) afterward, Medicare Part A will pay for up to 100 days of SNF rehab.
- **Observation.** Outpatient. Medicare Part B applies. Hospital stay still gets billed, but to Part B (with the deductible and 20% coinsurance you pay on outpatient services). And here's the trap: **if Mom is on observation status, her stay does NOT count toward the 3-day inpatient requirement for Medicare to cover the SNF afterward.**

Translation: Mom can spend 4 nights in the hospital, get discharged to a rehab facility, and find out a week later that Medicare won't pay a dime of the rehab bill because those 4 nights were technically "observation."

How much does this cost?

A 20-day SNF stay in NC averages \$300-\$450/day for the room, plus therapy charges. If Medicare doesn't pay because the 3-day inpatient threshold wasn't met, the family is on the hook for \$6,000-\$15,000 for that stay. Longer stays go higher.

I've seen this hit families for \$25,000+. They always say the same thing: "We thought Medicare covered everything."

What's the MOON form?

In 2017, Medicare started requiring hospitals to give patients (or their family) a **Medicare Outpatient Observation Notice (MOON)** within 24-36 hours of placing them on observation status. It's a form that says, in writing: "We're treating you as an outpatient under observation status. This may affect what Medicare pays."

Almost every family I've talked to never saw this form. They were either handed a stack of papers in the ER and signed without reading, or the form went into a folder nobody opened.

Action: The minute Mom is admitted, ask the nurse or case manager: "Is she inpatient or observation?" If they say observation, ask: "Where's the MOON form? Has it been given?" If you can advocate for

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inpatient classification before discharge, you can sometimes get it changed.

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The appeal window

A 2022 federal court ruling (Alexander v. Azar, now ongoing as Alexander v. Becerra) established that Medicare beneficiaries who were placed on observation status and later denied SNF coverage have the right to appeal that observation classification retroactively. The appeal must typically be filed within 120 days of the SNF coverage denial.

Action if you've already been hit: Don't pay the SNF bill yet. File the observation appeal first. Get help from your State Health Insurance Assistance Program (SHIP) — free counseling, every state has one. National SHIP locator: shiphelp.org.

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Most families I talk to believe Medicare covers 100 days of rehab. They're half right.

Here's what Medicare Part A actually covers for SNF stays:

Days	Coverage
1-20	Medicare pays 100% (no copay)
21-100	Medicare pays the room, but the patient owes a daily coinsurance — roughly \$220/day in 2026
101+	Medicare pays nothing. Full private-pay rate.

So the 100-day number isn't "100 days of free rehab." It's "100 days of partially-covered rehab, with an \$18,000+ patient coinsurance bill if you stay the full 80 days from day 21 to 100."

That's not the worst part. The worst part is that Medicare can stop paying for SNF *long before day 100* — even on day 5 or 10 — if the facility decides the patient is "no longer making meaningful improvement." This is the **"failure to improve" denial**, and it's incredibly common.

The "failure to improve" denial

Medicare's SNF benefit requires that the patient be:

1. Recovering from a 3-day inpatient hospital stay (see Gap 1),
2. Receiving "skilled" care (PT/OT/speech, IV antibiotics, wound care, etc.),
3. Making **measurable improvement** from the rehab, OR requiring skilled care to maintain function.

That third criterion is where families get hit. Therapists are required to document progress every few days. If the documentation shows a plateau, the facility's Medicare reimbursement gets cut. The facility then gives the family a **Notice of Medicare Non-Coverage (NOMNC)** — usually 2-3 days before discharge — saying Medicare will stop paying.

The family gets 48 hours to either accept the discharge or appeal. Most families accept because they don't know they can appeal.

What to do when you get the NOMNC

1. **Read the form immediately.** It tells you exactly how to file a fast-track appeal with the local Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO).
2. **Call the QIO number on the form within 24 hours.** The appeal is fast — they review the chart and rule within 72 hours.
3. **Stay in the facility while the appeal is pending.** Medicare keeps paying during the appeal. If you lose, you pay only for the days after the original NOMNC discharge date.

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Even if you lose, the appeal often buys an extra week of covered care. If you win, it can buy weeks more.

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The 2017 Jimmo settlement ~~nobody~~ talks about

There's a 2017 federal court settlement (Jimmo v. Sebelius) that explicitly says Medicare cannot deny SNF coverage solely because a patient has reached a "plateau." If the patient still needs skilled care to *maintain* function or prevent decline, coverage continues. Most family caregivers have never heard of Jimmo, and most facilities don't volunteer it.

Action: If a facility is pushing discharge because Mom "isn't improving," ask: "Are you applying the Jimmo standard? She still needs skilled care to prevent deterioration."

If Mom or Dad is on a Medicare Advantage plan (sometimes called Part C — UnitedHealthcare AARP, Humana, Aetna, Kaiser, etc.), the rules are different. Often dramatically worse.

Medicare Advantage plans must cover everything Original Medicare covers — by law. But they're allowed to require **prior authorization** before approving care. And they use it aggressively.

The prior auth numbers

Federal data from 2023 showed Medicare Advantage plans:

- Required prior authorization for **roughly 99% of SNF placements**
- Initially denied **about 13% of prior auth requests**
- Of those denied, **over 80% were overturned on appeal** — meaning the original denial was wrong

Read that again. The plan denies 13% of SNF requests upfront. When families appeal, the plan reverses itself 80% of the time. That tells you what those denials really are: a cost-control filter that bets most families won't push back.

What this looks like in practice

The hospital case manager calls the MA plan. The plan says: "We approve 5 days." Mom needs 21. Family gets the discharge notice on day 4. Family panics.

What most families don't know:

1. **You can appeal in real time.** The MA plan must respond to an expedited appeal within 72 hours (sometimes 24 if expedited urgent).
2. **The facility can't kick Mom out while the appeal is pending.** Federal rules require continuation of services during expedited appeals.
3. **The independent review entity (IRE) overturns these all the time.** Once the appeal escapes the plan's own internal review and goes to the IRE, denials get reversed at very high rates.

The MA "in-network" trap

MA plans also restrict which SNFs are in-network. The hospital might suggest a facility that's not in Mom's MA network — meaning the family either pays out-of-network rates (huge) or moves Mom to an in-network facility (logistics, family stress, sometimes a worse facility).

Action before the next hospital stay: Call Mom's MA plan today and ask for a list of in-network SNFs in her county and the adjacent counties. Keep that list. The day she's admitted, hand it to the discharge planner.

Should Mom switch back to Original Medicare?

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This is a real question more families should ask. The Medicare Annual Enrollment Period (October 15 - December 7) and the Medicare Advantage Open Enrollment Period (January 1 - March 31) are the windows. Switching has tradeoffs (drug coverage, supplemental insurance access, possible underwriting), but for a parent who's likely to need significant skilled care, Original Medicare is often more predictable than Medicare Advantage.

This is not a decision to make in a panic. It's a decision to make in October, with a SHIP counselor, looking at Mom's likely needs over the next year.

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This isn't a Medicare gap technically. It's a Medicaid gap. But it's the one that most often shows up in the same conversation — because Medicare runs out at 100 days, and Medicaid is what families turn to for long-term nursing home care.

Here's the problem: **You don't get to start qualifying for Medicaid the day you need it.**

When Mom applies for Medicaid to pay for a nursing home, the state reviews the past **60 months of her financial transactions**. If she gave away assets — to children, grandchildren, a trust, a charity — during that 60-month window, Medicaid imposes a **penalty period** of ineligibility.

How the penalty works

The penalty is calculated as: (value of transferred assets) ÷ (your state's monthly Medicaid nursing home rate) = months of ineligibility.

Example: Mom gave the family home (valued at \$250,000) to her son three years ago. North Carolina's Medicaid nursing home rate is roughly \$8,500/month in 2026. The penalty:

$\$250,000 \div \$8,500 = 29.4 \text{ months of ineligibility.}$

That means Mom can't get Medicaid coverage for 29 months from the day she applies. Who pays for the nursing home during those 29 months? The family does. Out of pocket. Roughly $\$8,500/\text{month} \times 29 = \$246,500$.

The son who got the house didn't save the family money. He cost them \$250K and lost the house, because typically the family ends up needing to sell it anyway during the penalty period to fund Mom's care.

"Just give the house to the kid" — why it backfires

This is one of the most expensive pieces of bad advice that gets passed around at family gatherings. It comes from a real planning strategy (long-term Medicaid planning *requires* a 5+ year runway), but families almost always apply it too late.

The strategy only works if Mom is genuinely healthy and at least 5 years from needing nursing home care. If she's 78 and starting to decline, the lookback is going to catch the transfer.

What actually works

Real Medicaid planning is its own specialty — too complex to fully solve here. But the principles are:

1. **The 5-year clock starts the day the transfer happens.** If Mom is healthy and you're considering this strategy, do it now, not next year. Every month delayed shortens the runway.
2. **Some transfers are exempt** — to a spouse, to a disabled child, to a caregiver child who lived with Mom for 2+ years and provided care that delayed institutionalization, to certain trusts. An elder law attorney can identify what applies.

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3. **Spend-down strategies are sometimes better than gifting** — paying off the mortgage, prepaying funeral costs, buying exempt assets (a primary residence if not currently owned).

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4. **Income trusts (Miller trusts in NC) handle the income side.** Most states impose an income cap on Medicaid eligibility, and a Miller trust can shelter excess income to qualify.

Action: If Mom or Dad is over 75 and the family is having any conversation about future care needs, schedule a one-time consult with an elder law attorney in your state. Not Mom's general estate planning lawyer — an elder law specialist. The consultation usually costs \$300-\$500. It can save the family six figures.

If a parent (or surviving spouse of a parent) is a wartime veteran, there's a Veterans Affairs benefit called **Aid & Attendance (A&A)** that can pay up to roughly \$2,800/month in 2026 (single veteran), \$3,300/month (married veteran), or \$1,800/month (surviving spouse). It can be used for in-home care, assisted living, or skilled nursing facility care.

It's tax-free. It's on top of Medicare. It's renewable as long as the qualifying conditions persist.

And it's massively underused. VA estimates suggest only about a third of eligible families ever apply.

Who qualifies (in plain English)

A parent qualifies for Aid & Attendance if all of these are true:

1. **They served at least 90 days of active military duty**, with at least one day during a recognized wartime period (WWII, Korea, Vietnam, Persian Gulf, post-9/11, etc.).
2. **They were discharged honorably** (or under conditions other than dishonorable).
3. **They need help with at least 2 activities of daily living** (bathing, dressing, eating, transferring, toileting, continence) OR are blind, OR are in a nursing home, OR have a 5/20 visual acuity or worse.
4. **Their income, minus recurring medical expenses, is below the VA's annual pension threshold.**
5. **Their net worth (excluding home and one car) is below roughly \$159,000 in 2026.**

The income test is what trips most families up. It's not just income — it's *income minus unreimbursed medical expenses*, including the cost of the in-home care or assisted living itself. So a parent paying \$5,000/month for assisted living often qualifies even if their income looks high on paper.

Surviving spouse qualifies too

A surviving spouse of a wartime veteran who never remarried can qualify for Aid & Attendance under the same medical and financial criteria. The spouse doesn't have to have served — the veteran did.

This is the most missed angle. Lots of families have a widowed mother who would qualify because her late husband served in Korea or Vietnam, but nobody ever asks the question.

How to apply

1. Get your parent's DD-214 (military discharge form). If you can't find it, request a copy from the National Archives at [archives.gov/veterans](https://www.archives.gov/veterans).
2. Get medical documentation of the ADL needs (from Mom's doctor — a simple letter listing the daily-living tasks she can no longer do independently).
3. Pull together income and asset documentation.

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4. **Do NOT use a "VA benefits planner" who charges an upfront fee.** Federal law prohibits charging for VA claim preparation. Use a VA-accredited attorney or a veterans service organization (VSO) — they help for free. Find one at va.gov/vgo/accreditation.asp.

5. File via va.gov or in person at the local VA office.

Processing takes 3-6 months on average. The award is retroactive to the application date, so file as early as possible even if some documentation is still pending.

Action: Right now, go ask Mom or Dad: "Did Dad ever serve? When?" If the answer is yes, this benefit is potentially \$30,000+/year of help that the family is not using.

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Most coverage gaps are created in the first 48 hours of a hospital stay. The family that knows what to do in that window saves \$10K-\$30K versus the family that finds out a week later. Here's the prep.

One-time prep (do this in the next 7 days)

- Find out exactly what Medicare Mom has. **Original Medicare** (with or without a Medigap supplement), or **Medicare Advantage** (which plan and which company). Photo of the card, saved on your phone.
- If she's on Medicare Advantage, call the plan and ask for the list of in-network skilled nursing facilities in her county and adjacent counties. Save it.
- Find out who Mom's local State Health Insurance Assistance Program (SHIP) counselor is. shiphelp.org → enter ZIP code. Save the number.
- Locate Mom's healthcare power of attorney (HCPOA) document. Make sure the person named is reachable and able to act. If no HCPOA exists, get one drafted this month.
- Find Dad's DD-214 if he was a veteran. Even if you don't apply for Aid & Attendance now, you'll want it within reach.
- If Mom is over 75 and has significant assets, consult an elder law attorney about the 5-year Medicaid lookback.

The moment Mom goes to the hospital

Within the first 24 hours, you (or the HCPOA holder) must ask the nurse or case manager four specific questions:

1. **"Is she inpatient or observation?"** Write down the answer with the date and time. If observation, ask when she was placed on observation status.
2. **"Where is the MOON form?"** If observation status applies, this form must be provided within 24-36 hours. Get a copy.
3. **"What's her admitting diagnosis, and what's the expected length of stay?"** This sets up the conversation about SNF need.
4. **"If she needs rehab afterward, are you planning to send her to a skilled nursing facility or home with home health?"** Find out early so you can verify in-network status if she's on MA.

If observation is wrong (she clearly needs to be inpatient based on the care being given), advocate. Talk to the attending physician, not just the nurse. The physician can change the order.

When the discharge planner enters

Discharge planners are usually thoughtful people working under time pressure. They will move Mom *out* of the hospital as soon as the chart says she's stable. That's their job.

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Your job is to make sure she's going to the right place:

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1. **If a SNF is recommended, ask:** Is this in her insurance network? Does the SNF accept her insurance? What's the expected length of stay? Has prior auth been submitted? When will they hear back?
2. **If home with home health is recommended, ask:** Has the home health agency been contacted? When will they start? What hours? Will Mom be safe alone in the gap between hospital discharge and home health start?
3. **Always ask for the discharge summary in writing.** Hospital discharge instructions are often rushed and contradicted within 48 hours of discharge.

When the NOMNC arrives (during a SNF stay)

If Mom is in a SNF and you get a Notice of Medicare Non-Coverage saying coverage will end in 48 hours:

1. Read it carefully. It explains your fast-track appeal rights.
 2. Call the BFCC-QIO number on the form within 24 hours to file the appeal.
 3. Stay in the facility while the appeal is pending.
 4. If the appeal is denied, you've still bought a few extra covered days.
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Federal & state programs

- **State Health Insurance Assistance Program (SHIP)** — free, unbiased Medicare counseling in every state. shiphelp.org
- **Medicare hotline** — 1-800-633-4227 (1-800-MEDICARE), 24/7
- **Medicare.gov** — for plan comparison, finding doctors, finding SNFs, comparing nursing homes (the Nursing Home Compare tool is excellent)
- **VA Aid & Attendance information** — va.gov/pension/aid-attendance-housebound
- **VA-accredited attorney/VSO locator** — va.gov/ogc/accreditation.asp

Appeals & advocacy

- **Center for Medicare Advocacy** — medicareadvocacy.org. Nonprofit, excellent self-help materials on observation status, SNF denials, and Jimmo settlement enforcement.
- **Medicare Rights Center** — medicarerights.org. Free helpline at 1-800-333-4114.
- **National Academy of Elder Law Attorneys (NAELA)** — naela.org, attorney directory for Medicaid planning consultations.

Legal documents you should have

- **Healthcare Power of Attorney (HCPOA)** for Mom
- **Durable Power of Attorney (financial)** for Mom
- **HIPAA authorization** — so hospitals can talk to you
- **Living will / advance directive**

Most states' HCPOA forms are free online. If Mom doesn't have these in place, do not wait. The 2 AM hospital call doesn't care.

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Sometimes — often, actually — the math doesn't work. Mom's care needs exceed what Medicare and her savings will cover. The conversation turns to selling the family home.

This is where my day job comes in.

When a senior home sale becomes the plan, the family's biggest leverage is **time and clarity**. Time, because rushed sales — especially to cash investors who knock on the door — leave \$30K-\$80K of equity on the table. Clarity, because most families have never run the actual net-proceeds math.

I built a free tool that does that math:

Plug in the estimated sale price, the agent commission, the seller-paid concessions, the loan payoff, and the closing costs. The tool returns what actually hits the bank account at closing — which is almost always \$40K-\$60K less than the listing price.

Knowing that number changes the conversation. Suddenly the family is comparing apples to apples: "If we sell traditionally, we net \$X. If we accept the cash offer that came in the mail, we net \$Y. The difference is \$35,000." That clarity is the difference between a panic sale and a calm decision.

If a Medicare-driven home sale is on your horizon — even 6 months out — run the numbers now while it's calm.

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This guide is one chapter of the playbook.

The full senior transition involves not just Medicare coverage, but housing decisions (aging in place vs assisted living), caregiver coordination, financial planning, and the family conversations that make all of it possible. Each one connects to the others.

If you're early in this journey and you want the full system — module-by-module — that's what the **Senior Transition Blueprint** is. \$47 for the Core version, \$297 for the Premium version with a personalized plan and a 60-minute call with me.

You can find it at <https://www.rigginsstrategicsolutions.com/senior-transition-blueprint>.

If you'd rather start with the free version, the **Simple Blueprint** at <https://www.rigginsstrategicsolutions.com/simple-blueprint> is a 21-page starter that maps the whole journey.

Either way — Medicare is one of the most expensive things a family can be wrong about. Now that you've read this guide, you're less likely to be the family that finds out about the gap from the bill.

Questions or stuck on a specific situation?

Reply to the email this guide came in. I read every reply personally and answer most within a day. If you've got a real situation playing out right now — Mom's in the hospital, the NOMNC arrived, the Medicaid application is sideways — say so. I'll point you to the right resource.

Ryan Riggins Riggins Strategic Solutions ryan@rigginsstrategicsolutions.com

Disclaimer: This guide is general education, not legal, financial, or medical advice. Medicare rules change. Your state's Medicaid rules vary. For decisions involving your specific situation, consult a SHIP counselor (free), an elder law attorney, a VA-accredited claim agent, or your own healthcare team.

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